



POLICY

Medication: Zuclopenthixol Acetate (Clopixol Acuphase®) Prescribing

Scope (Staff):	NMHS Mental Health (MH) Staff: Clinical
Scope (Area):	NMHS Mental Health (MH) Areas

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1. Aim

This document provides NMHS Mental Health clinical staff with the requirements for prescribing zuclopenthixol acetate and for monitoring patients administered zuclopenthixol acetate in order to ensure safe and effective treatment.

2. Background

Zuclopenthixol acetate is indicated for treatment of acute psychoses, including mania, and exacerbations of chronic psychoses, particularly when the patient is hostile or aggressive. As the sedative action usually begins at least two hours after administration it has no place in rapid tranquilisation or acute sedation.

Zuclopenthixol acetate is only for use in NMHS MH inpatient units. It should only be used with the approval of the Consultant Psychiatrist and must not be used in:

- Patients with known hypersensitivity to zuclopenthixol or thioxanthenes
- Patients who are antipsychotic naïve
- Patients who will accept oral medication
- Patients who are unconscious

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- Patients who are pregnant
 - Patients with acute alcohol, barbiturate or opioid intoxication
 - Patients with infection of unknown origin
 - Patients who have phaeochromocytoma, blood dyscrasias
 - Patients who have circulatory collapse, depressed level of consciousness due to any cause, coma, suspected or established subcortical brain damage.

Zuclopenthixol acetate must **never** be prescribed:

- On an “as required” (PRN) basis.
- In an attempt to hasten the antipsychotic effect of other antipsychotics.
- For rapid tranquilisation or acute sedation.
- As a “test dose” for zuclopenthixol decanoate depot.

Precautions

- Patients who are receiving other antipsychotics - may lead to over-sedation which is difficult to reverse
- Patients with hepatic and/or renal impairment
- Patients with cardiac disease
- Patients who are at risk of arrhythmia i.e. genetic predisposition, hypokalaemia, hypomagnesaemia, concomitant medications which may prolong QTc interval
- Take care if patient is physically struggling as this medication is dangerous if accidentally administered into a vein
- Patients sensitive to extra-pyramidal side effects (EPSE).

3. Risk

Patients may suffer adverse outcomes related to zuclopenthixol acetate if prescription and monitoring is not in accordance with this policy.

4. Principles

4.1 Indications

- Acute psychosis and acute mania where other treatment modalities have been ineffective
- Zuclopenthixol acetate should be reserved for those patients who:
 - have required repeated intramuscular (IM) PRN medication for agitation and arousal.
 - have had sufficient time for assessment of response and/or side effects to previously injected medications (minimum of 60 minutes for IM and 15 minutes for IV).
 - OR have previously received zuclopenthixol acetate and have shown good tolerability and response to it.

4.2 Following Consultant Psychiatrist Assessment of the Patient

- Consultant psychiatrist approval is required for each dose.
- Zuclopenthixol acetate must be prescribed as a single STAT (immediate) dose (50mg to 150 mg by IM injection)* on the “Once Only” section of the Western Australia Hospital Medication Chart (WAHMC). Females and the elderly may require lower doses of 25mg - 50mg. Elderly patients must not receive more than 100mg as a single dose.
- There is no such thing as a ‘**course**’ of zuclopenthixol acetate. The patient must be clinically assessed by a doctor before every administration. **STAT orders of zuclopenthixol acetate must not be written in advance.**
- Zuclopenthixol acetate should be injected slowly into a large muscle i.e. gluteal muscle or lateral thigh
- Repeat doses may be prescribed preferably every 2 to 3 days, if necessary (a repeat dose may be needed 24 hours after the first dose in some cases),
- The maximum accumulated dosage for consecutive doses should not exceed 400 mg and the total number of injections should not exceed 4. It is expected that one or two injections will be sufficient.
- Treatment should be for a maximum of 2 weeks before switching to an alternative zuclopenthixol formulation or another antipsychotic.
- The team clinical pharmacist can be contacted during business hours or the on-call pharmacist contacted after-hours for further advice on dosing of zuclopenthixol acetate.
- Consideration should be given to prescription of PRN anticholinergic agents in case of EPSE. Anticholinergics should not be started prophylactically.
- All other parenteral antipsychotics should be withheld for 24 hours when patients are receiving zuclopenthixol acetate, including medications with a PRN dose frequency.
- Co-administration of zuclopenthixol acetate with a benzodiazepine or antipsychotic short acting IM injections should not be routine practice. Effects of any previously injected medication should be assessed before administering zuclopenthixol acetate (minimum of 60mins post IM and 15mins for IV). In the event co-administration of a short acting IM injection and zuclopenthixol acetate is deemed necessary, consideration should be given to the risk of excessive sedation or other adverse effect. The rationale for combining and any requirement for increased monitoring must be documented.

4.3 Presentation

- 50mg/1mL or 100mg/2mL glass ampoules (clear, yellow viscous oil)

Do not confuse with zuclopenthixol decanoate (Clopixol Depot®) a **long-acting** intramuscular injection

4.4 Monitoring

The first injection is usually the most sedating. Sedation may initially be seen within 2 hours after injection, peaks after 12 to 36 hours and may last up to 3 days. The following must also be considered:

- Prior to injection, baseline temperature (T), Heart Rate (HR), Blood Pressure (BP), Respiration Rate (RR), Oxygen Saturation (O2 sat) and Electrocardiogram (ECG) should be considered best practice.
- Patients must be carefully monitored following each injection. Physiological observations must be recorded on the Observation and Response Chart (ORC) PMR 38, with an update to the PSOLIS care plan regarding frequency of observations:
 - After injection the patient will be visually monitored for level of consciousness at 15, 30, 45 and 60 minutes and observations recorded on the PMR 38. The patient should also be monitored for presence of adverse event eg. EPSE and laboured breathing.
 - Vital signs are to be taken hourly at 2, 3 and 4 hours post injection.
 - Thereafter vital signs will be monitored 2 hourly at 6, 8, 10 and 12 hours coinciding with the anticipated period of maximum sedation.
 - If vital signs remain stable, observations may be taken 4 hourly for the period up to 48 hours. The decision to go to 4 hourly observations must be made in discussion with a Medical Officer and documented in the integrated progress notes.
 - If vital signs fall outside parameters or there is visible physical deterioration, the NMHS MH Clinical Deterioration Procedure is to be followed.
 - Vital signs may be completed more frequently as clinically indicated. This may be particularly relevant at 12 hours post zuclopenthixol acetate (when sedation is at its most significant), or at 24 - 36 hours post dose (corresponding with peak serum levels).
 - An ECG should be taken and reviewed by medical staff at 24 and 48 hours post injection to ensure safety of the QTc interval.
 - See [Appendix 1](#) for table of recommended monitoring frequencies.
- If the patient is fully alert and ambulating but uncooperative and physiological observations cannot be completed then it must be documented in the Integrated Progress Notes (PMR7) that the patient was visually assessed but refused observations.
 - When patients are asleep, respiratory rate (RR) and function will be monitored on an hourly basis).

Attempts will be made to wake the patient if their respiratory rate is less than 10 breaths per minute, or if there is any evidence of respiratory distress or obstruction. A code blue must be initiated if there is any difficulty arousing the patient.

4.5 Maintenance Treatment

- Oral zuclopenthixol can be commenced 2 to 3 days after the last injection of zuclopenthixol acetate or zuclopenthixol decanoate can be given with the last injection.



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- Zuclopenthixol decanoate depot: give 200mg - 400mg of intramuscular zuclopenthixol decanoate depot concomitantly with the last injection of zuclopenthixol acetate. Zuclopenthixol acetate can be mixed with zuclopenthixol decanoate in the same syringe with consideration of the total volume to be injected as per guideline: [Medication: Intramuscular Administration of Long-Acting Antipsychotics](#).

5. Compliance and Evaluation

- 100% of all prescriptions of zuclopenthixol acetate will comply with this policy.
- Zero reports on Datix CIMS.
- A retrospective audit will be carried out annually by NMHS MH Pharmacy.
- Results will be reviewed by the NMHS MH Drugs and Therapeutics Committee and the MH Management Committee.

Appendix 1. Recommended frequency for observations post zuclopenthixol acetate (Acuphase®) injection*

Monitoring Guidelines	Observations	Date	Time	Comments
Baseline (Pre-Injection)	T, HR, BP, RR, O2 sat, ECG			
<i>Acuphase administered</i>	-			
15 minutes	(Visual obs)			
30 minutes	(Visual obs)			
45 minutes	(Visual obs)			
60 minutes	(Visual obs)			
2 hours	T, HR, BP, RR, O2 sat			
3 hours	T, HR, BP, RR, O2 sat			
4 hours	T, HR, BP, RR, O2 sat			
6 hours	T, HR, BP, RR, O2 sat			
8 hours	T, HR, BP, RR, O2 sat			
10 hours	T, HR, BP, RR, O2 sat			
12 hours	T, HR, BP, RR, O2 sat			
16 hours	T, HR, BP, RR, O2 sat			
20 hours	T, HR, BP, RR, O2 sat			
24 hours	T, HR, BP, RR, O2 sat ECG			
28 hours	T, HR, BP, RR, O2 sat			
32 hours	T, HR, BP, RR, O2 sat			
36 hours	T, HR, BP, RR, O2 sat			
40 hours	T, HR, BP, RR, O2 sat			
44 hours	T, HR, BP, RR, O2 sat			
48 hours	T, HR, BP, RR, O2 sat ECG			

*This table can be used as a prompt – all observations must be documented on **PMR 38**

Related internal policies, procedures and guidelines

[NMHS MH Clinical Deterioration Procedure](#)

[NMHS MH Physical Health Care Management Policy](#)

[NMHS MH Medication: Management of High Risk Medication](#)

References

- Zuclopenthixol acetate (Acuphase®) Product Information – eMIMS
- Taylor et al. The Maudsley Prescribing Guidelines in Psychiatry 14th Ed.
- Therapeutic Guidelines: Psychotropic Version 8, 2021
- South Metropolitan Health Service Zuclopenthixol Acetate (Clopixol Acuphase®) Guidelines
- [WA High Risk Medication Policy](#)

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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 8: Governance, leadership and management		☒ Std 10: Delivery of Care ☒ 10.1 Supporting Recovery ☒ 10.4 Assessment and Review ☒ 10.5 Treatment and Support		
Chief Psychiatrist's Standards for Clinical Care:	Assessment, Physical Health Care of Mental Health Consumers, Risk Assessment and Management				
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AUTHORISATION: (For MH policies only – delete this box if not applicable)

Committee Authorising	Date Authorised	Minutes Ref. No.	Document Reference (eg MHEG Meeting
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Document			Minutes)
MHEG	22/05/2013	151/13	Meeting Minutes
NMHS MH DTC	02/08/2016		DTC Minutes
NMHS MH STC	03/12/2019		DTC Minutes
MHEG	03/01/2020		Emails on W: drive

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Date	Version	Author	Notes
6/01/2020	V2.0	A/Chief Pharmacist	Updated template, monitoring doses included, cautions and precautions reviewed and updated.
7/12/2021	V3.0	NMHS MH A/Chief Pharmacist	Template updated, links; minor changes to information; recommended monitoring frequency form added.
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The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD1117.

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